PRODUCT DEMO · DRAFT, OCTOBER 2026

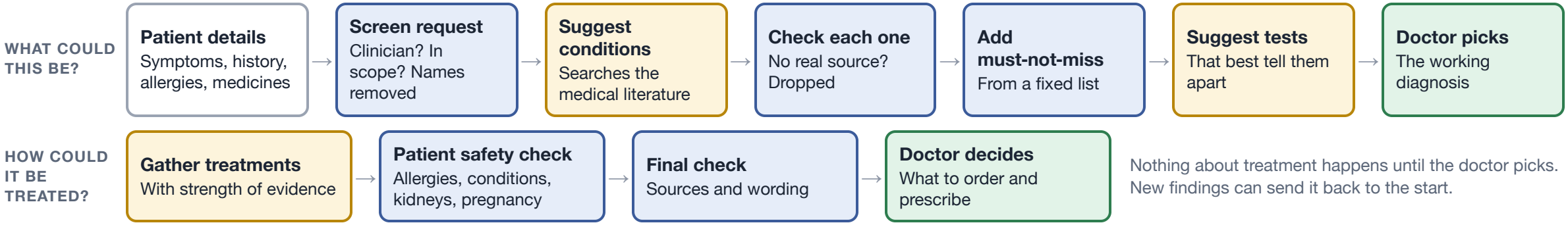
# Physician's Copilot

A research assistant for doctors: suggests possibilities, shows where every claim comes from, flags what must not be missed.

> The copilot **suggests, cites and flags.**
> The doctor **judges, orders tests and prescribes.**

## HOW IT WORKS: ONE FIXED ROUTE, WITH CHECKS THE AI CANNOT SKIP

AI researcher · Fixed check · Doctor




Nothing about treatment happens until the doctor picks. New findings can send it back to the start.

## THE PROBLEM

- A doctor has minutes to weigh possible conditions, the tests that separate them, and treatments safe for *this* patient.
- Nobody can keep up with the thousands of papers published every day.
- Rare but dangerous conditions get buried under "most likely".
- Today's AI chatbots sound sure and can invent sources.

## GUARDRAILS

- ✓ **No source, no claim**, and the source must actually say it. Withdrawn papers are never shown.
- ✓ **Looks for evidence against** each possibility, not only for it.
- ✓ **Must-not-miss conditions are always shown**, from a doctor-curated list, not AI judgement.
- ✓ **Drug warnings come from reference databases**; the AI can explain one but not switch it off.
- ✓ **The doctor decides**: treatment research waits for their pick.
- ✓ **Honest wording**, missing information listed, names removed before any outside search.

## NEXT STEPS

1. Pick the first setting, patient group and country.
2. Agree the must-not-miss list and who keeps it current.
3. License a drug-interaction source (the free one closed).
4. Build the full route and test it on made-up cases.
5. Recruit doctor reviewers; sign privacy agreements before real data.

Where we are: design agreed and medical reference library in place; guardrails are being built now.